COPAY CARD IMPACT · CALCULATOR

Free Drug. Full Price Claim. Your Plan Pays.

A 5-step walkthrough to model how manufacturer copay cards redistribute cost from member to plan, plus an accumulator vs. maximizer decision matrix. The math is usually bigger than employers expect.

01 The Five-Step Calculation

The copay card transaction is real money moving in a direction most employers have not traced. Run these five steps in order, with your PBM data and your member cost-sharing structure open. Step 3 is where the number usually lands harder than expected.

STEP 1 · DATA

Identify copay card utilization in your data.

Request from your PBM: drugs with active manufacturer copay card programs in your formulary, members using copay cards on each drug, and total claims dollars where copay assistance was applied. If your PBM cannot identify copay card utilization, document that answer; the inability to report on this is itself diagnostic.

STEP 2 · CURRENT STATE

Determine your accumulator status.

For each drug with copay card utilization, find out whether your plan uses an accumulator, a maximizer, or nothing. With no program, copay card amounts count toward the member's accumulators by default; the member hits OOP max faster, the plan pays everything sooner. That is your baseline.

STEP 3 · PER-DRUG MATH

Model the per-drug financial impact.

For each high-cost drug with copay card utilization, gather: annual drug cost per member, member copay or coinsurance per fill, copay card value per fill (often public from the manufacturer), fills per year, and the member's deductible and OOP max. Calculate how quickly the card pushes the member past OOP max. After that point, the plan pays 100%.

STEP 4 · PLAN-LEVEL AGGREGATE

Estimate plan-level impact.

Aggregate across every drug with a copay card program. Calculate the total additional plan cost from accelerated OOP max accumulation versus the scenario where members paid their cost-sharing out of pocket without a copay card. The difference is the cost redistribution from member to plan, in dollars.

STEP 5 · DECISION

Evaluate your options.

No accumulator or maximizer? Evaluate implementing one. Using an accumulator? Assess member impact when cards exhaust mid-year. Using a maximizer? Confirm your PBM is coordinating correctly. For any approach, identify the five to ten drugs with the largest copay card financial impact and start there.

Illustrative example for educational purposes. Actual amounts vary by plan based on drug mix, member population, and copay card programs in effect.

03 Accumulator vs. Maximizer · Decision Matrix

Once Step 5 surfaces the financial impact, the design choice is between three positions. Each has different implications for plan cost, member experience, and PBM administration complexity. There is no universal right answer; the right answer depends on your drug mix and member population.

POSITION A · NO PROGRAM

Most expensive scenario for the plan.

Copay card amounts count toward the member's deductible and OOP max by default. The member hits OOP max faster than they would without the card. The plan absorbs the full cost from that point through year-end. This is the baseline; if you have not actively chosen accumulator or maximizer, you are here.

POSITION B · COPAY ACCUMULATOR

Blocks the manufacturer's payment from counting.

The manufacturer keeps paying the copay, but the member's accumulators do not advance. When the card runs out, the member faces full cost-sharing. Some switch to generics. Others stop therapy. Saves the plan money; can disrupt members on specialty therapies for serious conditions. Best fit when the drug class has clinically equivalent alternatives.

POSITION C · COPAY MAXIMIZER

Restructures cost-sharing to capture full card value.

The member's cost-sharing is set at the maximum the card will cover; the card pays it. The member pays nothing out of pocket. The plan avoids the cost it would otherwise bear under Position A. Preserves member experience but requires more sophisticated PBM administration; not every PBM coordinates this correctly.

HYBRID · INCREASINGLY COMMON

Maximizer on most categories, accumulator with carve-outs.

Recent designs apply maximizers across most copay-card-eligible drugs while running accumulators on categories where alternatives are clearly available, with carve-outs for therapeutic areas where discontinuation risk is highest (oncology, transplant, severe autoimmune). Requires a capable PBM and clear clinical criteria for the carve-out list.

04 Implementation Watch-Outs

The math points to a position. The implementation determines whether the plan actually captures the projected savings without disrupting members. Four common failure modes to plan around before flipping the switch:

§ **Member communication 60 days before go-live**
Members on copay-card-supported therapies need to understand what is changing and what their out-of-pocket exposure will be when the card exhausts. One letter is never enough; build a multi-touch communication plan with prescriber outreach for the highest-volume drugs.

§ **PBM execution capability check**
Maximizer programs depend on the PBM's ability to coordinate cost-sharing structures with manufacturer card values in real time. Confirm your PBM has done this implementation before, ask for client references, and require performance guarantees in writing. Not every PBM does this well.

§ **Carve-out list aligned with clinical criteria**
If running a hybrid, the carve-out list (drugs and conditions where the accumulator does not apply) should be built from clinical criteria, not from a marketing list the PBM provides. Engage your clinical advisory team in defining the carve-outs.

§ **Quarterly performance review**

Track copay card utilization, member discontinuation rates, and plan savings quarterly for the first year. Patterns surface fast; if discontinuation in a specific category exceeds expectations, expand the carve-out list before the second year locks in.